

Lyfe Place Abuja

Business case.

SUMMARY

A private ambulatory medical campus.

A four-structure residential compound in Abuja converted into a medical park under Medbury's Medical Infrastructure Division. Senior specialists hold sessions there without taking a lease. Families buy bundled care. Aesthetic and day-case surgery anchors the identity. A laboratory, imaging and a pharmacy sit on site, so a patient is consulted, worked up, treated and dispensed to in a single visit and goes home the same day.

Asset	711 sqm gross across four structures, 519 sqm of usable rooms
Capacity	5 sessional consulting rooms, a day-case theatre, a treatment room, a hair transplant suite, three imaging modalities, a laboratory and a pharmacy
Investment	NGN 647M as costed, NGN 526M on a reduced fit-out
Return	NGN 517M campus revenue and NGN 273M a year to Medbury at stabilisation
Payback	2.4 years from stabilisation, 1.9 on the reduced fit-out
Ramp funding	NGN 45M at its deepest, cash positive in month 9

01 / THE ASSET

What we have.

Structure	Gross sqm	Rooms sqm	Use
Main building, ground	286	182	Arrival, theatre suite, conversion clinic, imaging
Main building, first	277	239	The consulting plaza, hair transplant, treatment room
Guest chalet	96	61	Laboratory and specimen handling
Boys' quarters	52	37	Pharmacy: dispensary, retail, controlled drugs, cold chain
Campus	711	519	73% of gross is usable room

Ground floor, 182 sqm of rooms

Room	sqm	Zone	Note
Theatre	24.7	Theatre suite	Level trolley access, HEPA ventilation, isolated power
Monitored recovery, 2 bays	13.0	Theatre suite	Adjoins the theatre
Sterile store and dirty utility	7.8	Theatre suite	One-way dirty to clean
Conversion clinic, consulting 1	19.3	Conversion clinic	
Conversion clinic, consulting 2	18.2	Conversion clinic	Existing water and drainage
Digital X-ray, lead shielded	14.1	Campus	Cannot be sited elsewhere
Phlebotomy draw point	10.4	Campus	Specimens routed to the chalet
Medication collection point	6.0	Campus	Serving hatch to the waiting area
Reception, waiting, concierge	58.1	Common	Two waiting zones, scrub recess
Patient WCs	10.4	Common	

The theatre suite is ring-fenced as a sole-use zone entered off the lobby. The conversion clinic and the campus diagnostics sit outside it.

First floor, 239 sqm of rooms

Room	sqm	Zone	Note
Consulting rooms 1, 2 and 3	51.6	Sessional pool	New partitions off a central spine
Consulting room 4	19.8	Sessional pool	En-suite WC
Consulting room 5	19.8	Sessional pool	En-suite WC
Consulting room 6	22.4	Family practice	The bundled-care service
Hair transplant suite	24.3	Anchor	All-day single-patient cases
Treatment room	14.8	Anchor	Local anaesthesia, existing water and drainage
Ultrasound and echocardiography	13.7	Campus	Beside the consultants who order it
Phlebotomy draw point	5.7	Campus	So patients do not descend for bloods
Doctors' and members' lounge	34.0	Common	Part of what membership buys
Sterilising, linen, WC, terrace	32.6	Common	

Five rooms make up the sessional pool, once the theatre, the conversion clinic, the family practice and the anchor have their space. At four bands a day over six days that is 5,520 half-day blocks a year, and it is the ceiling on how much the campus can let.

02 / THE OFFER

What we sell.

Line	Sold to	What it is	Revenue
Panel and membership	Specialists	Access to rooms and an address without a lease	Fees and sessions
Family practice	Families and corporates	Bundled annual care with quarterbacked referral into the panel	Packages
Day case and aesthetics	Cash-pay patients	Aesthetic medicine and surgery, dermatology, hair transplant	Theatre and suite fees
Conversion clinic	Alameda	Two ground-floor rooms and campus services	Licence
Diagnostics and pharmacy	All of the above	Laboratory, imaging, dispensing, retail	Per test, per script

The family practice is what makes the rest work. A panel of specialists with no patients is an empty building. Bundled care generates first contact, refers into the panel, and drives diagnostics and pharmacy volume. It is the only line the campus controls end to end.

Sessions

Band	Length	Member	Panel or guest
Early, 07:00 to 09:00	2 hours	44,000	66,000
Morning, 09:00 to 13:00	4 hours	61,000	91,500
Afternoon, 13:00 to 17:00	4 hours	61,000	91,500
Evening, 17:00 to 21:00	4 hours	94,000	141,000
Treatment room	per session	180,000	270,000
Day-case theatre	per half-day list	750,000	1,125,000

Anaesthetist fees are billed through to the operating clinician with a coordination margin, not carried by the campus.

Membership ladder

Tier	Launch	Mature	Session rate	What it buys
Panel	200,000	400,000	Guest rates	Credentialing, directory listing, booking access
Associate	600,000	1,200,000	20% off guest	Pays for itself at 0.8 sessions a week
Full	1,500,000	2,400,000	35% off guest	Priority booking, named room preference, lounge
Fellow	2,500,000	4,200,000	35% off guest	A standing weekly slot, directory prominence, inclusion in campus marketing

Tier	Launch	Mature	Session rate	What it buys
Group	1,200,000 each	2,000,000 each	35% off guest	Three or more from one practice. Shared named room, pooled blocks, one invoice
Partnership	Revenue share	Revenue share	n/a	For anchor lines. The campus provides room, equipment and marketing, the clinician provides the practice

Launch rates run for the first two years and are held for three years for anyone who joins in year one. That is the reason to join early, and it costs less than an empty diary costs. Mature rates apply to everyone joining from year three, at which point the address is established and the ladder pays for itself on usage: Associate at 0.8 sessions a week, Full at 1.2, Fellow at 2.2.

Family practice packages

Package	NGN a year	What is included
Individual	600,000	Unlimited GP access, annual screen, vaccinations, chronic disease management, quarterbacked referral
Family, up to four	1,500,000	As above for the household, with paediatric cover
Corporate, per head	400,000	Executive screen, occupational health, priority access

03 / THE MARKET

Who buys it, and what is evidenced.

The base case requires the following. Each is stated so it can be checked rather than assumed.

What the base case needs	Meaning	Status	How it gets settled
25 members across the tiers	Specialists committing annually	Not yet evidenced	Survey in the field
35 panel specialists	Credentialed, booking at guest rates	Not yet evidenced	Survey in the field
41% room fill	2,270 half-day blocks sold a year	Borrowed from comparable Lagos practice	Survey in the field
150 family practice members	Households and corporate heads	Not yet evidenced	Needs its own panel model
25% theatre fill	138 half-day lists a year	Judgement for a new day-case theatre	Anchor clinician discussions
13,300 patient contacts	Across all lines, about 53 arrivals a day	Within the single-reception ceiling of 98	Arithmetic, not assumption

How large the pool actually is

Sizing the specialist market	Count
Consultants in the FCT with meaningful private practice	300 to 600
In specialties an ambulatory plaza suits, needing no theatre or beds	x 60%
Who would pay for a private address rather than use hospital rooms	x 15 to 25%
Addressable	36 to 72
Converted within two years at 30% to 50%	12 to 36

The base case needs 60 names in total, 25 of them paying members. That sits inside the addressable pool rather than consuming it, and the low case at 32 names is comfortably within two years of ordinary conversion.

Membership uptake and room fill together carry most of the revenue, and neither is yet evidenced. A survey is in the field with Abuja consultants testing session demand by time band, willingness to pay, and membership pricing. Forty responses will settle both. Nothing else in the plan is cheaper to test or more consequential if wrong.

04 / THE OPERATOR

Who runs it.

Party	Role	Scope
Medbury Healthcare	Owner	Holds the head lease, the fit-out and the campus. Owns the diagnostics and pharmacy businesses. Carries the capital and the operating cost
Mezo Health	Manager	Runs the campus day to day: bookings, front office, patient navigation, billing and collection, tenant relations, compliance and reporting
Consult for Africa	Delivery	Diagnostics phase, interior design and fit-out delivery. Hands over to Mezo at practical completion

Management fee	Basis	Note
Base fee	6% of campus revenue	Paid monthly
Incentive	10% of gross operating profit above a threshold	Rises only when the campus performs
Cap	10% of campus revenue combined	Protects the owner if profit runs ahead of revenue
On-site staff	Engaged by Mezo, reimbursed at cost	Front office, nursing, records
At base case	NGN 51.7M a year	8.9% of campus revenue

05 / THE INVESTMENT

What goes in.

Capital, NGN M	As costed	At 70%
Head rent, two years in advance	100.0	100.0

Capital, NGN M	As costed	At 70%
Agency, legal and caution deposit at 15%	15.0	15.0
Building fit-out	232.0	162.4
Theatre suite	120.0	84.0
Power, 25KVA hybrid solar	33.0	33.0
Medbury Diagnostics unit	28.0	19.6
Medbury Pharmaceuticals unit	24.0	16.8
Working capital, six months	95.0	95.0
Total capital	647.0	525.8

Release it in two phases

Fit-out and power amortise over ten years. The campus is expected to hold the building well beyond the initial term, and equipment can move to another balance sheet within the group if it does not. Phasing the release is about capital efficiency rather than accounting: there is no reason to expose the whole sum before the campus is trading.

Phase	NGN M	What it covers
Phase 1, on the two-year certainty already agreed	470	Rent, fees, working capital, power, the diagnostics and pharmacy units, and enough building fit-out to open and trade
Phase 2, released once a longer term is documented	177	The balance of the building fit-out, and the whole theatre suite
Total	647	Unchanged. What changes is when NGN 177M is exposed

Phase 1 trades on its own. Without the theatre it earns NGN 414M of campus revenue and returns NGN 205M a year on NGN 470M, a 2.3 year payback. The theatre carries the least certain demand on the campus, so it is the natural thing to hold until the surgical anchor is signed rather than built on expectation.

The second column carries every fit-out line at 30% below the costed figure. It is a reasonable target on builder's work, finishes and sourced furniture, which are about 46% of the building package. It is far less likely on the theatre, which is **88% equipment**: an air handling unit, an anaesthetic machine and an isolated power panel have hard prices and do not negotiate down by a third.

Building fit-out

Package	As costed	At 70%
Cooling, about 28 split units and cassettes	24.0	16.8
ICT: cabling, network, CCTV, access control, emergency call, room status	15.0	10.5
Electrical: boards, solar and grid zoning, medical circuits, emergency lighting	14.0	9.8
Joinery: reception, nurse stations, room casework	14.0	9.8
Interior decoration: painting, window treatments, soft finishes	12.0	8.4
Fire: detection, alarm, escape lighting	9.0	6.3
Clinical flooring, welded and coved vinyl	9.0	6.3
Ceilings and lighting	9.0	6.3
Partitioning to form the consulting rooms	8.0	5.6
Procedure room: scrub and clinical services	7.0	4.9
Signage and wayfinding	7.0	4.9
Mechanical fresh air and extract	6.0	4.2
Plumbing: clinical hand-wash, condensate	6.0	4.2
Doors and ironmongery	5.0	3.5
Medical waste holding and route	3.0	2.1
FF&E:: consulting rooms, reception, waiting, lounge	30.0	21.0
External: car park, lighting, drainage, gate house	10.0	7.0

Package	As costed	At 70%
Water: treatment, storage, pressure, hot water	5.0	3.5
Design, project management and approvals	14.0	9.8
Contingency at 12%	25.0	17.5
Building fit-out	232.0	162.4

Theatre suite

Package	As costed	At 70%	Nature
Ventilation: AHU, HEPA, ductwork, external plant	32.0	22.4	Equipment
Theatre lighting, pendant, operating table	32.0	22.4	Equipment
Anaesthetic machine and full monitoring	20.0	14.0	Equipment
Scrub, sterile store and dirty utility fit-out	15.0	10.5	Builder's work
Isolated power supply panel and UPS	11.0	7.7	Equipment
Recovery, two monitored stations	10.0	7.0	Equipment
Theatre suite	120.0	84.0	

06 / THE RETURN

What comes out, at stabilisation.

NGN M a year, stabilised	Low	Base	High
Members, paying tiers	12	25	35
Panel specialists	20	35	45
Family practice members	100	150	200
Sessional blocks sold	1,130	2,270	3,150
Room fill	20%	41%	57%
Membership and panel fees	31.4	65.0	91.8
Sessions	97.2	191.4	262.5
Treatment room	29.8	29.8	29.8
Day-case theatre	nil	103.5	132.5
Family practice packages	85.0	127.5	170.0
Campus revenue	243.4	517.2	686.6
Payroll, front office, nursing, records	(32.0)	(45.0)	(45.0)
Family practice: physicians, nurse, administration	(28.0)	(45.0)	(45.0)
Power	(24.0)	(28.0)	(28.0)
Theatre running: scrub, ODP, consumables	nil	(18.0)	(18.0)
Security, cleaning, waste, insurance, internet	(14.0)	(15.0)	(15.0)
Maintenance, biomedical, marketing, concierge	(16.0)	(16.0)	(16.0)
Mezo management fee	(14.6)	(41.1)	(68.2)
Head rent	(55.0)	(55.0)	(55.0)
Amortisation over ten years	(20.8)	(38.5)	(38.5)
Campus contribution	39.0	215.7	357.9
Diagnostics and pharmacy, net of staff	19.6	57.4	101.4
Total to Medbury	58.6	273.1	459.4

Costs total NGN 301.6M a year at the base case, which is NGN 25.1M a month. Of that, NGN 13.9M is operating cost, NGN 4.6M rent, NGN 3.4M the management fee and NGN 3.2M amortisation, which is not cash.

	Low	Base	High
Capital committed	470	647	647
Payback from stabilisation	8.0 yrs	2.4 yrs	1.4 yrs
On the reduced fit-out	372	526	526
Payback from stabilisation	6.3 yrs	1.9 yrs	1.1 yrs
Break-even, share of campus revenue	84%	58%	48%

The low case is Phase 1 only. A campus that recruits slowly does not release Phase 2, so there is no theatre to build, to run, or to amortise, and the capital at risk is NGN 470M rather than NGN 647M. Costs flex with it: a quieter campus runs fewer front-office shifts and one family physician rather than two, and the laboratory is not staffed for volume that has not arrived. Marketing is the one line that holds, because a slow start is exactly when it is needed.

The conversion clinic licence is excluded from all three cases. If it proceeds it adds approximately NGN 60M a year and is the only contracted lease line in the model.

07 / THE PATH

How we get there, and what it costs to.

Stabilisation is 24 months from opening. The campus loses money for part of that, and this is what has to be funded in the meantime.

Quarter from opening	Ramp	Cash in	Cash out	Net	Cumulative
Pre-opening	-	-	25.0	(25.0)	(25.0)
Q1	15%	21.9	38.4	(16.5)	(41.5)
Q2	25%	36.5	39.8	(3.3)	(44.8)
Q3	40%	58.4	41.7	16.6	(28.2)
Q4	55%	80.3	46.5	33.7	5.6
Q5	70%	102.2	48.5	53.6	59.2
Q6	82%	119.7	48.1	71.6	130.8
Q7	92%	134.3	49.4	84.8	215.6
Q8	100%	145.9	50.5	95.4	311.0

The campus turns cash positive in month 9 and repays the whole ramp by month 12. The deepest point is **NGN 45M**, against NGN 95M of working capital. Two things make that work. **The two years of rent paid in advance cover the entire 24-month ramp**, so no rent cash leaves the business before stabilisation. And NGN 209M of the NGN 584M stabilised revenue is memberships and family practice packages, which are annual and collected up front, so the money arrives ahead of the use it pays for.

If the ramp disappoints

Ramp speed	Funding needed, NGN M	Cash break-even	Against NGN 95M provision
Brisk, 100% by month 24	45	Month 9	Comfortable
Slow, 82% by month 24	71	Month 12	Inside the provision
Very slow, 66% by month 24	93	Month 15	Inside, but almost no headroom

Term and payment are different questions

Two years is what has been discussed and what the agreement is drawn up on. A five-year term is a fresh conversation to be raised, and for now the sensible step is to communicate the intent and obtain verbal approval rather than to reopen a settled document. Medbury does not need to, and should not, prepay five years in any case. **The term drives the amortisation; the payment schedule is a separate matter.** Two years up front and annually thereafter is the right shape, whatever term is eventually agreed.

One event to plan for: rent becomes a cash cost for the first time in month 25. The prepayment runs out exactly as the ramp ends. At stabilisation the campus generates about NGN 95M a quarter net, so NGN 13.75M of quarterly rent is absorbed comfortably, but it is a step and it should not arrive as a surprise.

Cash out during the ramp excludes rent, which is prepaid, and amortisation, which is not cash. Marketing runs at double the stabilised rate until the campus reaches 80% of base case, because demand has to be built before it can be served.

08 / THE RISKS

What could stop it.

Risk	Level	What it is	What reduces it
Demand	High	Membership uptake and room fill carry most of the revenue and neither is evidenced	Survey in the field. Ten founding members signed before fit-out completes would settle a quarter of it
Cost	Medium	Fit-out is the largest line and has not been measured	Quantity surveyor's take-off before commitment. The reduced column shows the downside is manageable
Concentration	Medium	The evening band and memberships together are a large share of revenue and rest on the same proposition	If the survey finds evening demand thin, both fail together. Test them as one question, not two
Capacity	Medium	The building caps at about 78% fill and 98 arrivals a day	Growth beyond the high case needs more rooms, not more effort. Plan the second site rather than over-promise this one
Regulatory	Medium	Theatre registration, radiation licence, practitioner credentialing	All are process rather than obstacle, but each has a lead time and none can be compressed
Key clinician	Medium	The anchor lines depend on named individuals	Partnership terms rather than session lets, so the clinician is invested in the campus rather than renting from it
Lease term	Medium	The agreement drawn up is for two years and a longer term has yet to be raised	Communicate the intent early and secure verbal approval. Phase the capital so the larger commitments follow the conversation

Conditions precedent, in the order they bind

Condition	Why
1 Pay the two years as agreed, on time	The agreement is drawn up and the terms are settled. Prompt payment is also what earns the standing to ask for anything further: a longer term cannot be negotiated from behind on rent
2 Raise the five-year intent and obtain verbal approval	A fresh conversation, not a variation of what is signed. Verbal comfort is enough to proceed with Phase 1. It must be documented before Phase 2 capital is released
3 Change of use approval and the landlord's written consent	Before the NGN 115M of rent and fees is paid
4 A quantity surveyor's take-off	Before fit-out capital is released
5 Consultant demand evidenced	Forty survey responses, or ten founding members signed
6 Facility registration and radiation licence	Registration to reflect surgical and sedation capability. NNRA licence before imaging installs
7 A written transfer agreement	With a hospital holding theatre and intensive care capability. A licensing expectation and an insurance condition

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Room areas are measured from the as-built drawings and are to be confirmed against the CAD. Fit-out is built up by package and requires a quantity surveyor's take-off. Session rates are derived from comparable Lagos practice. Membership uptake, room fill and family practice panel size are assumptions the consultant survey is designed to test. Not a binding offer, and not legal or tax advice. FX reference USD/NGN 1,550.